

# Four receptors. Nine molecules.

GLP-1

GIP

amylin

glucagon

the receptor family the whole field is built on

# 587%

growth in US prescriptions for GLP-1  
receptor agonists, 2019 to 2024.

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Two molecules are approved for obesity.  
Seven more are in trials.

Receptor targets, stage and indication. No trade names.  
Sources on the final slide. As at August 2026.

## HOW THEY WORK

# One target became four.

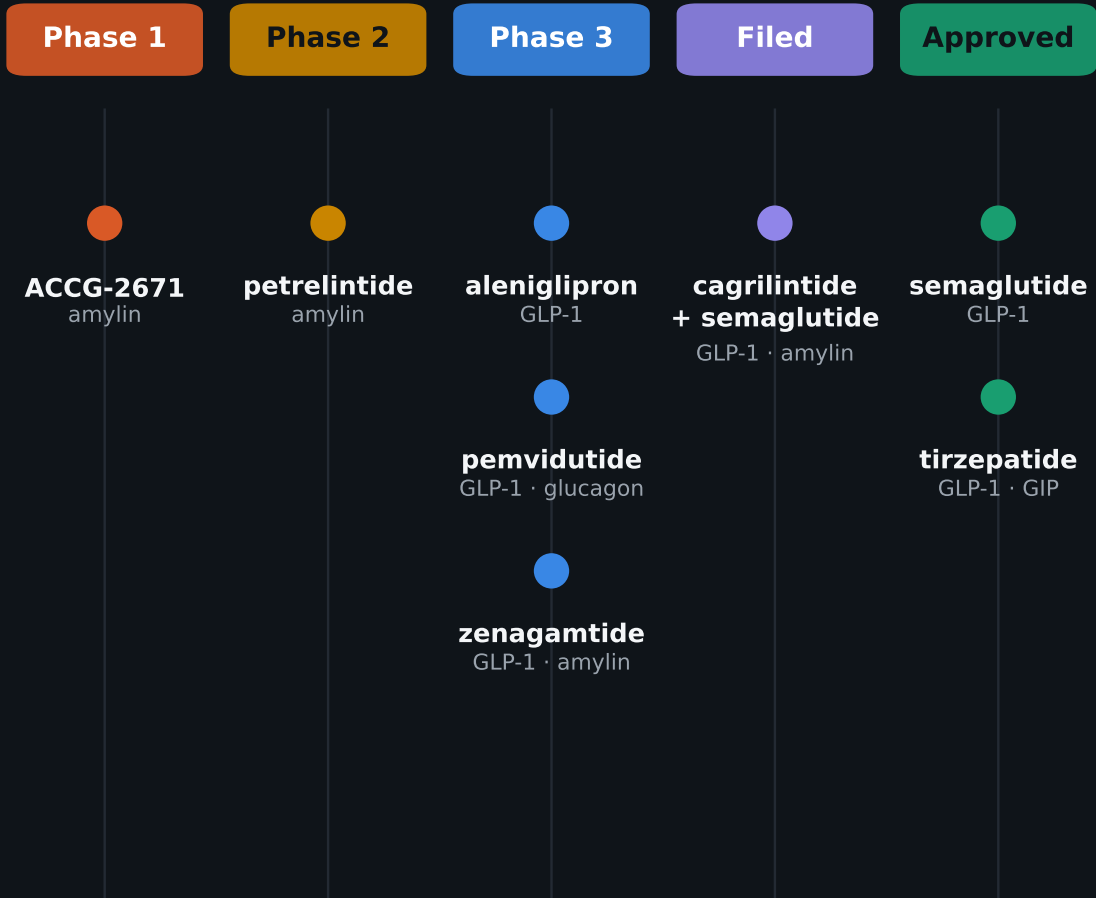
|                            | GLP-1 | GIP | amylin | glucagon |
|----------------------------|-------|-----|--------|----------|
| semaglutide                | ●     | ●   | ●      | ●        |
| tirzepatide                | ●     | ●   | ●      | ●        |
| cagrilintide + semaglutide | ●     | ●   | ●      | ●        |
| aleniglipron               | ●     | ●   | ●      | ●        |
| pemvidutide                | ●     | ●   | ●      | ●        |
| zenagamtide                | ●     | ●   | ●      | ●        |
| petrelintide               | ●     | ●   | ●      | ●        |
| ACCG-2671                  | ●     | ●   | ●      | ●        |

The first medicines hit one receptor. The newer ones pair GLP-1 with a second — to add effects, not just dose harder.

A filled dot means the molecule targets that receptor.

## WHERE EACH ONE STANDS

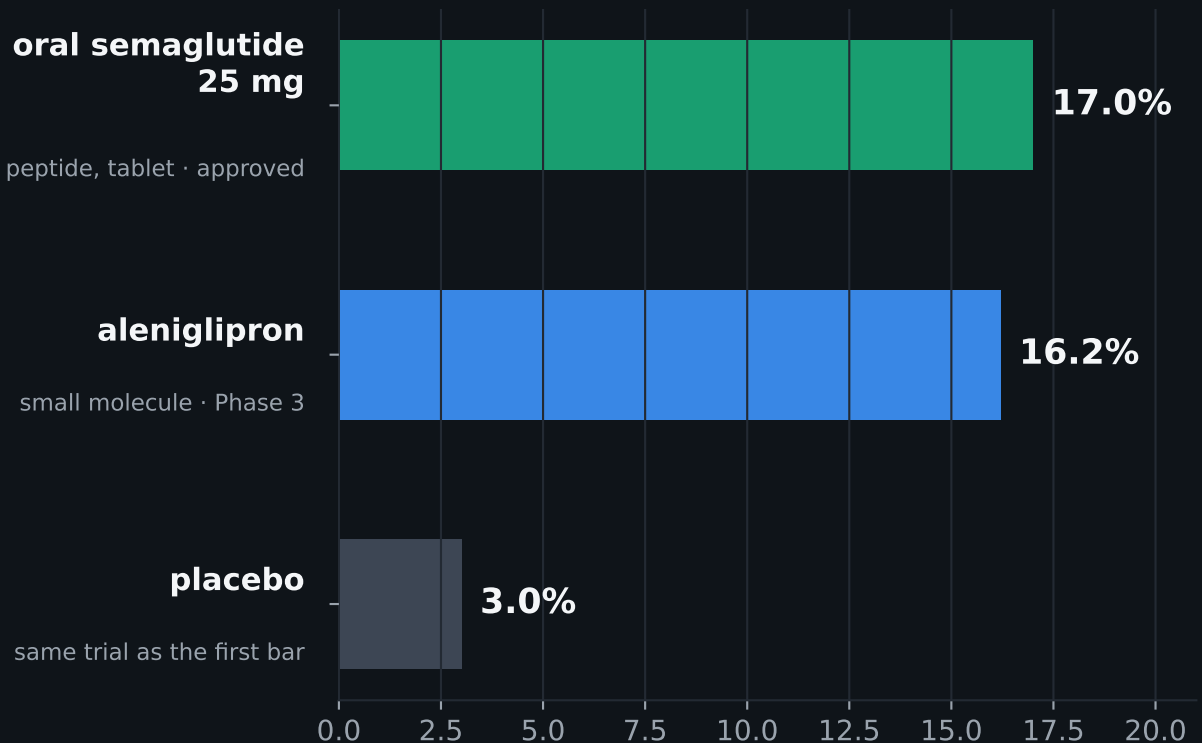
# Two on the market. Seven behind them.



Only two mechanisms have reached patients. Everything else — oral formats, combinations, new receptors — is still being tested.

Stage as at August 2026. Development stages change; several readouts are due.

# The next contest is oral.



A tablet changes who can realistically be treated, not just how much weight comes off. One is a peptide in tablet form; the other a small molecule, simpler to manufacture at scale.

Not a head-to-head comparison. Different trials, populations and durations: 17% versus 3% placebo at Phase 3b; 16.2% is the upper end of an open-label extension, with a 10.4% discontinuation rate.

# The same biology is moving into the liver.

## Obesity / overweight

semaglutide

tirzepatide

cagrilintide  
+ semaglutide

aleniglipron

zenagamtide

petrelintide

ACCG-2671

## Liver disease (MASH)

semaglutide

pemvidutide

## Alcohol use disorder

pemvidutide

Adding glucagon acts directly on the liver while GLP-1 handles appetite. That widened the field from weight to liver disease, and now to addiction.

One molecule is approved in MASH, one of only two treatments for it. A glucagon/GLP-1 agonist began Phase 3 there in Aug 2026; data due 2029.

# What this covers.

Receptor targets, approval status and trial stage as at August 2026, from company filings, company releases and industry landscape reviews.

No trade names. No share prices, valuations or company comparisons — this is a view of the science, not of the market.

Trial results are point-in-time and stages change. Cross-trial figures are not head-to-head.

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## Sources

SEC filings (10-Q, 8-K) and company releases, Dec 2025 – Aug 2026; regulatory announcements; industry landscape reviews.

## Related work

[namikakmandev.github.io](https://namikakmandev.github.io)

Not medical or investment advice.